

Specification of uncertain clinical inputs

(symptoms, context, missingness model)



Uncertainty profile $u(x)$

(information / diagnostic overlap / policy)

Decision control signal



Triage-first safety gate + TiTrATE logic

(rule-based clinical reasoning)



Decision-behaviour groups (G1–G5)

Care pathways + rationale

Screening and routing, not diagnosis

XAI-by-design artefacts

(traceability, auditability)

Governance and monitoring

(safety metrics, regression tests, audits)